

Critical Care Cheat Sheet 2026

AAA (1)	- Emergent surgery - Admission/Transfer - IV BP med control
Acidosis	pH <7.25
Airway (1)	- IM/IV Epi - Admit/transfer/ICU - Intubation - Epiglottitis - Angioedema - Croup (multiple racemic nebs with distress) - Ludwig's - RPA - Smoke Inhalation - Esophageal Perforation - Partial or Complete Airway Obstruction
Anaphylaxis (1)	- Hypotension - IM/IV Epinephrine (document re-evaluations)
Anemia (1) Hgb <8	- HR >120 - BP <80 - Sx: Syncope, Palps, CP, SOB - PRBC/Platelets
Arterial Occlusion (1)	- Immediate Surgery - Transfer - IV anticoagulation
A Fib (1) Tachy >60 min OR New onset HR >150	- Continuous IV meds - Vent Rate >150 with treatment - Sx: CP, SOB or Lightheadedness
Asthma/COPD (1)	1 continuous neb or 3 respiratory treatments (in distress) - NIPPV / IPPV - IV Mag - O2 <80% on usual O2 - RR >32 - Acidemia (document re-evaluations & pre/post treatment spO2)
Bradycardia (1) (symptomatic)	- Pulse <40 - Pacer - Atropine - Cards c/s for transvenous pacing
Cardiac Arrest	30+ min of post resuscitation care (CPR itself is a procedure)
Cardiac Tamponade (1)	- Hypotension despite IV Bolus - Pericardiocentesis

Chest Pain (2)	- Nitro Drip - Transfer to Cardiac Center or Cath Lab - TPA / Heparin / Lovenox - New EKG Changes (STE/ST dep/ischemia) - CT to look for other causes of pain - Pulmonary Edema - Elevated Troponin
CSpine Fracture (1)	- Admission / Transfer - Deficit (weak, paresth)
CHF (1)	- BiPAP (pCo2 >60) - IV Nitroprusside, Nitro, Dobutamine, Dopamine - Multiple doses IV Diuretics
Compartment Syndrome (1)	- Emergent surgery - Fasciotomy in ED - Transfer
Dehydration	BP < 80 Systolic w/ 1+ L IVF
DKA (1)	- pH < 7.25 - ICU admit - insulin gtt
Dysrhythmia (1)	- V Tach: IV meds or cardiovert - V Fib: always CC 3rd degree Block: always CC - Torsades: IV Mag - External Pacemaker or Synchronized Cardioversion - Symptomatic SVT or AFib RVR - Meds: IV Atropine, Lidocaine, Amiodarone, Procainamide
Ectopic Pregnancy (1)	- To OR / Admission / Transfer - Blood Transfusion in ED
Epidural Abscess	Always CC
Eye	Acute Glaucoma: visual changes and treated aggressively RAO/CVO: ophtho consult
Fracture	Open due to Trauma: Femur, Pelvis, Tibia, Humerus
GI Bleed (1)	- SBP <80 - HR >120 - ED scope, ICU, Surgery, EGD or Colonoscopy w/in 12 hours - PRBC, plasma or platelet - Multiple fluid boluses - Pressors - Hgb <7 - AMS
GCS	12 or below always CC (Except hypoglycemia)
Hemothorax Pneumothorax	>25% and/or management of chest tube
Hypercalcemia	Requiring Admission

HTN Emergency (1) Systolic >230 OR Diastolic >130	- End organ damage: CVA, ARF, NSTEMI, AMS, CHF/pulm edema - Chest pain - Pre-Eclampsia/Eclampsia - IV Meds - GTT: Nitro, Nipride, Cardene
Hypokalemia	- K <2 AND IV K administered - ECG changes
Hypoglycemia	3+ Accucheck w/ intervention 3+ doses of D50 OR D10 gtt
Hypothermia	Temp <94.1F or 33C
Hypotension (1) (Adult)	- SBP < 80 - RR > 32 3L or more IVF - IV Pressors - CVL
Hypoxia / Hypoxemia	O2 <80% on usual O2
Hyperkalemia	HyperK Protocol (Ca2+, Bicarb and/or Insulin and D50)
Hyponatremia (1)	- Admit / Transfer - Seizure
ICH / Stroke Intercerebral, subdural, subarachnoid, epidural	- TPA considered or given - Transfer / Admission - Intubation - Code Stroke even if negative
Meningitis	Bacterial → always CC Viral → w/ Encephalopathy
Mesenteric Ischemia	Always CC
MI (1)	- STEMI - NSTEMI - Elevated Trop (not chronic) - Emergent Cath - TPA
MVC (1) - AMS / GCS <12 - Abnl Vitals: O2 <85%, HR >120, SBP < 80	(1) - To OR / Transfer to trauma - Multiple IVF boluses - PRBC or Platelet transfusion - CVL - Spine, Pelvis, Femur Fx - Flail chest - PTX w/ chest tube - Intracranial bleed - Intubation - Solid Organ Injury
Neonatal Fever	<30d + Fever >100 + Full septic work up including LP

Ovarian Torsion Testicular Torsion	Always CC
Overdose	- Tylenol → IV / PO NAC - Digoxin → IV Digibind - Ethylene Glycol → Formeprizole (Antizol) - Salicylate → Emerg Dialysis - Snake Bite → IV Crofab
Pneumonia (1)	- O2 < 80% on usual O2 - HR > 120 A / HR >150 P - SBP <80 A / SBP <60 P - RR >32 A / RR >40 P
Pre-Eclampsia Eclampsia	- Admitted - Mag gtt - Seizure
Pulmonary Edema (1)	- IV Nitro - BiPAP or Intubation
PE / Extensive DVT	Treated with heparin or lovenox OR documented high risk of decompensation
Renal Failure (1)	- Emergent dialysis (AEIOU) - Vitals: HR >120, SBP <80 - HyperK: (Meds: Bicarb, Ca2+, IV Insulin, D50; ECG Changes: peaked T, Wide QRS) - Pulmonary Edema
Status Epilepticus	2+ doses antiepileptic (including EMS dose) - Continuous seizure without returning to baseline x 20 min
Sepsis (2) SBP <80 = always CC	- Multiple IVF Boluses 2+ IV Antibiotics - AMS - HR >120 - Temp >104 OR <91.4 - Lactic >4
Psych Suicide Attempt (1)	- Abnormal VS / medically ill - Serious attempt to harm (hanging, neck wound, gsw, etc) - Multiple IM/IV meds for agitation to reduce risk of harm to patient or others
Thyroid Storm Myxedema Coma	Treated with IV Beta Blockers Treated with IV Levothyroxine
Trauma (1)	- AMS - Threat to life or limb - Transfer to higher facility - Stab/GSW to neck, abdomen or chest +FAST exam - Perforated viscous